Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 0.1 | **Date:** 04-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Phase III, Randomised, Double-Blind Study to Assess the Efficacy, Safety and Tolerability of Baxdrostat in Combination With Dapagliflozin Compared With Dapagliflozin Alone on Chronic Kidney Disease (CKD) Progression in Participants With CKD and High Blood Pressure **Short Title/Acronym:** Baxdrostat + Dapagliflozin in CKD and HTN **Protocol Number:** D6972C00003 **NCT Number:** NCT06268873 **Posting ID:** 582265794866287747 **Sponsor Name:** AstraZeneca **Investigational Product:** Baxdrostat in combination with Dapagliflozin **Phase of Development:** Phase III **Medical Monitor:** AstraZeneca Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To determine whether baxdrostat/dapagliflozin compared with dapagliflozin alone slows CKD progression and reduces the risk of End-Stage Kidney Disease (ESKD). **Secondary Objectives:** To determine whether baxdrostat/dapagliflozin is superior to dapagliflozin alone at reducing UACR (urine albumin-creatinine ratio), at reducing systolic blood pressure (SBP), and to determine whether the combination slows the rate of kidney function decline following the hemodynamically-mediated acute effect on Glomerular Filtration Rate (GFR).

2.2 Background & Rationale To address the significant risk of disease progression in patients with chronic kidney disease (CKD) who also suffer from difficult-to-control high blood pressure (hypertension). Dapagliflozin is a proven SGLT2 inhibitor that protects kidney health, while baxdrostat is an aldosterone synthase inhibitor. Research suggests that utilizing baxdrostat in combination with dapagliflozin can effectively and safely lower blood pressure and protect kidney function in patients already receiving standard-of-care ACE inhibitors or ARBs.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Dapagliflozin + placebo matching baxdrostat) **Blinding:** Double-blind **Randomization:** Randomised **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING)

3.2 Planned Enrollment & Duration Target N: 2455 **Number of Sites:** Global multi-center trial **Estimated Study Start:** 29-Mar-2024 **Estimated Primary Completion:** 24-Feb-2028

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants ≥ 18 years of age. 2. Confirmed diagnosis of CKD with an eGFR ≥ 30 and < 90 mL/min/1.73 m2 at screening, and a Urine albumin creatinine ratio > 200 mg/g and < 5000 mg/g. 3. History of hypertension with a Systolic Blood Pressure (SBP) ≥ 130 mmHg at screening and ≥ 120 mmHg at randomisation, managed on a stable, maximum tolerated dose of an ACE inhibitor or an ARB (not both) for at least 4 weeks prior to screening.

4.2 Exclusion Criteria 1. Systolic blood pressure > 180 mmHg, or Diastolic Blood Pressure (DBP) > 110 mmHg at screening. 2. Known hyperkalaemia, defined as serum potassium ≥ 5.5 mmol/L within 3 months at screening. 3. Any use of mineralocorticoid receptor antagonists (e.g., spironolactone, eplerenone, finerenone), potassium-sparing diuretics, or potassium binders within 4 weeks prior to screening. 4. Type 1 Diabetes Mellitus (unless meeting specific regional criteria with prior SGLT2i use) or uncontrolled Type 2 Diabetes Mellitus (HbA1C $> 10.5\%$).

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Daily administration of baxdrostat (with protocol-defined up-titration criteria to higher doses) and dapagliflozin (ATC Code: A10BK01; Trade names include Qternmet, Qtern, Xigduo, Farxiga). **Route of Administration:** Oral (tablets). **Duration of Treatment:** A 4-week dapagliflozin run-in period, followed by a 24-month double-blind treatment period, and subsequently a 6-week open-label period of dapagliflozin alone.

5.2 Concomitant & Prohibited Therapy Permitted: Stable maintenance therapy of an ACE inhibitor or ARB. **Prohibited:** Concurrent use of other SGLT2 inhibitors (except study protocol dapagliflozin), mineralocorticoid receptor antagonists, potassium-sparing diuretics, potassium binders, or systemic immunosuppressive therapy.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Up to -28 Days	Informed Consent, Medical History, Baseline eGFR, BP & Labs
Run-in	4 Weeks	Single-blind dapagliflozin administration to establish baseline
Randomisation	Day 0	Randomisation to Baxdrostat+Dapagliflozin or Placebo+Dapagliflozin
Interim	Weeks 2, 4, 8, 16, then Q4 Months	Onsite Visits: Safety Labs (focus on potassium/sodium), BP checks, Efficacy Assessment, IP Accountability
End of Study	Month 24 to Week 6 Post-	

treatment | Final Double-Blind Vital Signs, Transition to Open-Label Dapagliflozin, Final eGFR checks |

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Kidney hierarchical composite endpoint, defined as the most severe outcome of the following: 1. Death; 2. Kidney Failure Replacement Therapy (KFRT - chronic dialysis or kidney transplant); 3. Sustained GFR < 15 mL/min/1.73 m2; 4. Sustained GFR decline from baseline of $\ge 57\%$; 5. Sustained GFR decline from baseline of $\ge 50\%$; or 6. individual change from baseline to post-treatment eGFR. **Measurement Method:** Central Laboratory Analysis and mortality/hospitalization tracking.

7.2 Secondary & Exploratory Endpoints * Change from baseline in urine albumin-creatinine ratio (UACR). * Change from baseline in systolic blood pressure (SBP). * Change in eGFR following randomisation to post-treatment to measure the slowed rate of kidney function decline.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular study visits and central lab analyses. Particular attention paid to serum potassium levels to monitor for hyperkalemia and sodium levels to monitor for hyponatremia. **Serious Adverse Events (SAEs):** Reported to the sponsor within 24 hours of site awareness. **Data Monitoring Committee (DMC):** External independent committee reviewing unblinded safety and efficacy data.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy analyses, Safety Set for all subjects who receive at least one dose of the investigational product. **Sample Size Justification:** Appropriately powered ($\alpha = 0.05$, $Power = 80\%$) to detect statistically significant differences in the kidney hierarchical composite endpoint and systolic BP changes between active and active-comparator arms. **Handling of Missing Data:** Multiple Imputation (MI) and model-based estimations for continuous longitudinal data such as eGFR and BP changes.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits according to a risk-based monitoring framework. **Audit Trail:** Robust Electronic Data Capture (EDC) system with verified eCRF locking, source data verification, and data clarification forms.

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11. Study Identifiers & Governance

Primary ID: D6972C00003 **Registry Number:** NCT06268873 **Posting ID:** 582265794866287747 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** A Phase III Study to Investigate the Efficacy and Safety of Baxdrostat in Combination With Dapagliflozin on CKD Progression in Participants With CKD and High Blood Pressure **Official Regulatory Title:** A Phase III, Randomised, Double-Blind Study to Assess the Efficacy, Safety and Tolerability of Baxdrostat in Combination With Dapagliflozin Compared With Dapagliflozin Alone on Chronic Kidney Disease (CKD) Progression in Participants With CKD and High Blood Pressure

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12. Clinical Framework (Trial Specific)

Primary Condition: Chronic Kidney Disease and Hypertension (Preferred Name: Chronic kidney disease due to hypertension) **Preferred UMLS Name:** Hypertensive Chronic Kidney Disease **SNOMED ID:** 104931000119100 **Diagnosis Threshold:** eGFR ≥ 30 and < 90 mL/min/1.73 m2; SBP ≥ 130 mmHg **Medical Methodology:** Combined Aldosterone Synthase Inhibitor (ASI) and SGLT2 Inhibitor therapy **Optimization Target:** Preservation of kidney function (eGFR trajectory) and targeted reduction in Systolic Blood Pressure

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13. Study Procedures & Methodology

Management Pathway: Sequential introduction of SGLT2 inhibitor (dapagliflozin) followed by randomisation to ASI (baxdrostat) addition. **Education Protocol (HCP):** Site initiation visits with comprehensive protocol training regarding up-titration criteria, potassium monitoring, and AE management. **Education Protocol (Patient):** Onsite training emphasizing medication compliance, recognizing signs of hypotension/hyperkalemia, and visit schedule adherence. **Quality Audit Mechanism:** Source data verification of blood pressure logs and central laboratory values vs. eCRF inputs.

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14. Observation & Follow-Up Schedule

Total Duration: ~ 26.5 months total (4-week run-in, 24-month double-blind, 6-week open-label) **Onsite Visit Cadence:** Weeks 2, 4, 8, 16, and every 4 months thereafter **Remote Monitoring Frequency:** Intermittently between prolonged onsite gaps, based on site protocol **Checklist Review Interval:** Every 4 months (at onsite visits) during the maintenance phase

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]